

ECHS FA-05 — Repeat Admission Fraud Report

Repeat Admission Fraud Analytics · FY 2025-26

FA-05 | Rule-Based Signal Detection

CLASSIFICATION	PERIOD	RECORDS ANALYSED	QUERY SETS
RESTRICTED	01-Apr-2025 to 31-Mar-2026	7,276,987 Claims	6 Signals / 1,639,312 Beneficiaries

ECHS — Airawat Research Foundation

21 May 2026

ECHS Repeat Admission Fraud Screening

FY 2025-26 · 1 April 2025 – 31 March 2026

Report date: 21 May 2026 **Methodology:** Rule-based signal detection across six independent fraud indicators.

Note to reviewer: This report is produced by an automated screening system. Every flagged case is a *lead for investigation*, not a confirmed finding. A qualified auditor should review each case before any action is taken.

About This Analysis

This report presents the findings of **ECHS FA-05 — Repeat Admission Fraud Analytics**, a systematic screening exercise applied to all ECHS inpatient claims for the financial year 2025-26.

The analysis uses six independent rule-based checks, each designed to detect a specific pattern that is known to be associated with fraudulent or irregular repeat-admission billing. These checks were designed based on established health insurance audit frameworks and adapted for the ECHS claims data structure.

Each check operates independently. A beneficiary may be flagged by one check or by several simultaneously. Cases appearing in multiple checks are the highest investigation priority, because each flag represents a separate dimension of anomaly that is unlikely to arise by coincidence.

The checks cover the following patterns:

- **Unusually short gaps** between consecutive hospital admissions for the same patient
- **Repeated admissions** for the exact same non-chronic diagnosis
- **Simultaneous or back-to-back stays** at two different hospitals
- **Very high total admission counts** for a single beneficiary within the year
- **Hospital-level patterns** where a facility systematically re-admits patients within 30 days at rates far above the national average
- **Escalating claim amounts** for successive admissions for the same condition

Data Overview

Metric	Count	Notes
Total claims in scope (all types)	7,276,987	FY 2025-26, all admitted beneficiaries
Inpatient (IPD) claims	1,290,898	17.7% of total — primary scope for all signals
Unique beneficiaries in scope	1,639,312	Distinct ECHS card holders with at least one claim
Unique beneficiaries flagged (any signal)	174,168	10.6% of all beneficiaries in scope
Hospitals screened (S5)	593	Hospitals with ≥30 IPD claims in the year

Key Findings — Beneficiary Risk Profile

Of the **1,639,312** beneficiaries with claims in scope, the composite risk distribution is as follows:

Composite Risk Level	Beneficiaries	As % of All Beneficiaries	What This Means
● High	71,777	4.38%	Flagged by 4+ signals, OR 2-3 signals where at least one is Critical — immediate audit recommended
● Medium	49,667	3.03%	Flagged by 2-3 signals, OR a single Critical-level signal — review in next audit cycle
● Low	52,813	3.22%	Flagged by exactly one Medium or High signal — monitor; include in sample audit
Total flagged	**174,257**	**10.63%**	

99,329 beneficiaries were flagged by two or more independent checks — these are the highest-priority cases for investigation.

Severity guide: ● Critical — Immediate investigation · ● High — Priority review within two weeks · Medium — Next scheduled audit cycle

Flagged Cases — Sample

For each check: 5 Critical · 3 High · 2 Medium representative cases.

Check 1 — Early Re-admissions

Gap between a patient’s discharge and their next IPD admission at any hospital. **Scope:** 149,216 patients / 368,100 pairs

Risk Level	Flagged	Threshold
● Critical	167,024	Re-admitted within ≤7 days of discharge
● High	84,925	Re-admitted 8–15 days after discharge
● Medium	116,151	Re-admitted 16–30 days after discharge

Patient Name	Hospital	First Admitted	Discharged	Re-admitted After	Same Hospital	Risk Level
SUNDER SINGH	METRO HOSPITAL & HEART INSTITUTE - NOIDA	30-May-2025	09-Jun-2025	0 days	Yes	● Critical
CHANDRA SINGH	UJALA CYGNUS CENTRAL HOSPITAL	18-Mar-2026	22-Mar-2026	0 days	No	● Critical
AMAR SINGH	RAMADEVI MEDICAL CENTRE PRIVATE LIMITED	21-Sep-2025	23-Sep-2025	0 days	No	● Critical

Patient Name	Hospital	First Admitted	Discharged	Re-admitted After	Same Hospital	Risk Level
SUBE SINGH	HUMAN CARE MEDICAL CHARITABLE TRUST	05-May-2025	05-May-2025	0 days	No	● Critical
MUNESH KUMARI	MEENAKSHI JAIN HOSPITAL	20-Apr-2025	22-Apr-2025	0 days	No	● Critical
JASVIR SINGH	Orthonova Joint & Trauma Hospital Pvt. Ltd.	22-Mar-2026	—	8 days	No	● High
BIRVATI	VELMED HOSPITAL	26-Sep-2025	03-Oct-2025	8 days	No	● High
UDAI SINGH	METRO HEART INSTITUTE - FARIDABAD	08-Apr-2025	19-Apr-2025	8 days	Yes	● High
VIJAY PAL	ASIAN INSTITUTE OF MEDICAL SCIENCES - FARIDABAD	18-Jul-2025	19-Jul-2025	16 days	Yes	● Medium
AMAR CHAND	MAX SUPER SPECIALITY HOSPITAL - MOHALI	22-Aug-2025	23-Aug-2025	16 days	Yes	● Medium

Check 2 — Same Diagnosis Repeated

Same non-chronic primary ICD-10 diagnosis appearing in multiple IPD admissions. **Scope:** 3,077 patients *Excludes 126 chronic ICD-10 prefixes — cancer, dialysis, transplant, etc.*

Risk Level	Flagged	Threshold
● Critical	150	4 or more admissions with the same diagnosis
● High	325	3 admissions, OR 2 admissions at different hospitals
● Medium	2,851	2 admissions at the same hospital

Patient Name	Diagnosis	No. of Admissions	Hospitals Involved	First Admitted	Last Admitted	Risk Level
BIKRAM RAY	Abdominal and pelvic pain	21	1	03-Apr-2025	08-Jan-2026	● Critical
MD STELLA MARRY	Symptoms, signs and abnormal clinical and laboratory findings, not elsewhere classified	14	1	02-Apr-2025	04-Mar-2026	● Critical

Patient Name	Diagnosis	No. of Admissions	Hospitals Involved	First Admitted	Last Admitted	Risk Level
MIRNAL KUMAR SENGUPTA	Symptoms, signs and abnormal clinical and laboratory findings, not elsewhere classified	14	1	10-Apr-2025	05-Mar-2026	● Critical
MUNIYAPPA	Symptoms, signs and abnormal clinical and laboratory findings, not elsewhere classified	11	1	16-Dec-2025	04-Mar-2026	● Critical
PRATIMA GHOSH	Symptoms, signs and abnormal clinical and laboratory findings, not elsewhere classified	10	1	08-Dec-2025	30-Jan-2026	● Critical
MAHENDRA SINGH	Diseases of liver	3	2	19-Jul-2025	30-Jan-2026	● High
PHOOL CHAND GURJAR	Unspecified acute lower respiratory infection	3	1	22-Nov-2025	07-Mar-2026	● High
SALWINDER SINGH	Septicaemia, unspecified	3	1	02-Jul-2025	20-Jan-2026	● High
IBRAHIM KHAN	Senile incipient cataract	2	1	16-Sep-2025	14-Oct-2025	● Medium
ASHOK KUMAR	Senile cataract	2	1	08-Dec-2025	22-Jan-2026	● Medium

Check 3 — Dual Hospital Stays

Same beneficiary admitted at two different hospitals at the same time or within 1 day of each other. **Scope:** 24,148 patients / 28,730 pairs

Risk Level	Flagged	Threshold
● Critical	4,775	Same-date admission or overlapping stays
● High	23,955	Hospital B admission within 1 day of Hospital A discharge

Patient Name	Hospital A	Admitted (A)	Discharged (A)	Hospital B	Admitted (B)	Overlap Type	Risk Level
Y BABU PAILA	CONTINENTAL HOSPITALS PRIVATE LIMITED, HYDERABAD	09-Apr-2025	12-Apr-2025	DR AGARWALS EYE HOSPITAL(REP BY DR AGARWALS HEALTH, HYDERABAD	09-Apr-2025	Same-day dual admission	● Critical
PAKHAR SINGH	SRI GURU H ARKRISHNA SAHIB (C) EYE HOSPITAL TRUST - SOHANA, MOHALI	01-Apr-2025	01-Apr-2025	Fortis Hospital - Mohali, Mohali	01-Apr-2025	Same-day dual admission	● Critical
E THANUMALAYAN PILLAI	HOLY CROSS HOSPITAL - NAGERCOIL, NAGERCOIL	01-Apr-2025	01-Apr-2025	Dr Jeyasekharan Hospital And Nursing Home, Nagercoil	01-Apr-2025	Same-day dual admission	● Critical
PATTAN HAMEED KHAN	SUNSHINE HOSPITALS(A UNIT OF SARVEJANA HEALTH CARE PVT LTD), SECUNDERABAD	16-Apr-2025	18-Apr-2025	Secunderabad Main, Secunderabad	16-Apr-2025	Same-day dual admission	● Critical
MOHAR SINGH	PULSE MULTISPECIALITY HOSPITAL AND RESEARCH CENTRE, KOTPUTLI	01-Apr-2025	01-Apr-2025	MANIPAL HEALTH ENTERPRISES PRIVATE LIMITED, JAIPUR	01-Apr-2025	Same-day dual admission	● Critical
PUSHPA	DR AGARWALS HEALTH CARE LIMITED, SATARA	07-Apr-2025	07-Apr-2025	SATARA HOSPITAL & RESEARCH CENTRE PVT LTD, SATARA	09-Apr-2025	Back-to-back (<=1 day)	● High
SHASHIKALA	SADHU VASWANI MISSIONS MEDICAL COMPLEX, PUNE	08-Apr-2025	14-Apr-2025	Aditya Birla Health Services Ltd, Pune	14-Apr-2025	Back-to-back (<=1 day)	● High
BHAGWATI DEVI SHARMA	GURUKRIPA HOSPITALS - SIKAR, SIKAR	01-Apr-2025	05-Apr-2025	MANIPAL HEALTH ENTERPRISES PRIVATE LIMITED, JAIPUR	05-Apr-2025	Back-to-back (<=1 day)	● High

Check 4 — High Admission Frequency

Total number of separate IPD admissions by a single beneficiary during the year. **Scope:** 114,289 patients

Risk Level	Flagged	Threshold
● Critical	26,747	6 or more admissions
● High	27,307	Exactly 5 admissions
● Medium	60,235	3 or 4 admissions

Patient Name	Total Admissions	Different Hospitals	First Admitted	Last Admitted	Avg Days Between	Risk Level
GURDIAL SINGH	177	2	01-Apr-2025	18-Jul-2025	1 days	● Critical
TILAK RAJ	155	3	02-Apr-2025	30-Mar-2026	2 days	● Critical
MOHAR SINGH KAPOOR	155	3	02-Apr-2025	29-Mar-2026	2 days	● Critical
SUBHASH KUSHWAH	154	1	01-Apr-2025	29-Mar-2026	2 days	● Critical
NAR SINGH	150	1	03-Apr-2025	28-Mar-2026	2 days	● Critical
DHARAM SINGH	5	4	13-Apr-2025	06-Mar-2026	82 days	● High
SUDHIR KUMAR SINGH PAL	5	2	25-Jun-2025	12-Jan-2026	50 days	● High
SUNITA DEVI	5	4	08-Jun-2025	19-Nov-2025	41 days	● High
JITENDRI DEVI	4	1	09-May-2025	11-Feb-2026	93 days	● Medium
MOHAN SINGH	4	1	05-May-2025	18-Dec-2025	76 days	● Medium

Check 5 — Hospital Re-admission Pattern

A hospital's 30-day re-admission rate compared to the national median. **Scope:** 593 hospitals *Only hospitals with ≥30 IPD claims in the year are included.*

Risk Level	Flagged	Threshold
● Critical	129	Re-admission rate >3× national median
● High	176	Rate 2–3× national median
● Medium	288	Rate 1.5–2× national median

National median 30-day re-admission rate: **10.76%**

Hospital	City	Total Claims	Re-admission Rate	vs National Median	Avg Days to Re-admit	Risk Level
CANCER RELIEF SOCIETY RST REGIONAL CANCER HOSPITAL	NAGPUR	55	100.00%	9.3x	12 days	● Critical

Hospital	City	Total Claims	Re-admission Rate	vs National Median	Avg Days to Re-admit	Risk Level
KRISHNA CANCER HOSPITAL	BHOPAL	40	100.00%	9.3x	19 days	● Critical
BUDDHA CANCER CENTRE PVT. LTD.	PATNA	372	80.39%	7.5x	12 days	● Critical
ASSAM CANCER CARE FOUNDATION (ALL HOSPITAL)	GUWAHATI	535	75.82%	7.0x	12 days	● Critical
HCG PANDA CANCER HOSPITAL	CUTTACK	406	75.47%	7.0x	17 days	● Critical
PUNJAB CANCER CARE AND MULTISPECIALITY HOSPITAL	BATHINDA	724	32.24%	3.0x	13 days	● High
MAXIVISION EYE HO PVT LTD - HYDERABAD	HYDERABAD	280	32.28%	3.0x	9 days	● High
MAX SUPER SPECIALITY HOSPITAL - SHALIMAR BAGH	DELHI	951	32.16%	3.0x	13 days	● High
SWAMI RAMA HIMALAYAN UNIVERSITY	DEHRADUN	3,423	21.49%	2.0x	14 days	● Medium
SHANTI SAROJ EYE HOSPITAL	MIRAJ	90	21.43%	2.0x	13 days	● Medium

Check 6 — Claim Amount Escalation

Growth in claim amount for the same diagnosis from the first admission to the most recent. **Scope:** 325 patients *Excludes first claims below ₹10,000 and the same 126 chronic conditions as Check 2.*

Risk Level	Flagged	Threshold
● Critical	138	Latest claim $\geq 3\times$ the first claim
● High	235	Latest claim $2-3\times$ the first claim
● Medium	10	Latest claim $1.5-2\times$ the first claim

Patient Name	Condition	Admissions	Claim Grew By	Same Hospital	Risk Level
LALAN SINGH	Diseases of the circulatory system	2	48.3x	Yes	● Critical
MEENA DEVI	Diseases of the circulatory system	2	47.5x	Yes	● Critical

Patient Name	Condition	Admissions	Claim Grew By	Same Hospital	Risk Level
MADAN LAL	BILATERAL OSTEOARTHRITIS OF THE KNEE (RIGHT LEFT)	2	37.0x	Yes	● Critical
KARNAIL SINGH	Chronic ischaemic heart disease / ACS - NSTEMI CAG DVD WITH ISR IN RCA (ADVICE PLAN 2 DEB TO RCA WITH CUTTING BALLOON) CCS GRADE IV POST PTCA TO LAD LVEF 48%	2	23.4x	Yes	● Critical
BISHESWAR NATH SINGH	Acquired deformity of chest and rib	2	20.3x	Yes	● Critical
NIRMAL PRADHAN	Other specified disorders of eye and adnexa in diseases classified elsewhere	2	3.0x	Yes	● High
GURMEET KAUR	Septicaemia, unspecified	2	3.0x	Yes	● High
JAGJIT KAUR	HYPOTHYROIDISM	2	3.0x	Yes	● High
VEER KAUR	Septicaemia, unspecified	3	2.0x	No	● Medium
TRILOK SINGH	Diseases of liver	4	1.9x	No	● Medium

Generated by ECHS FA-05 automated screening system.